

إسم المريض رابعياً : نجلاء رمضان عبدالهادي سليمان

المحافظة : القاهرة

رقم البطاقة : 27109220100581 رقم التليفون : 01113537878

العمر(تاريخ الميلاد) : 22-09-1971 المهنة : ربه منزل

العنوان : عمارات كلية البنات

تاريخ تسجيل الإستمارة : 02-10-2026

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد و

پیرتوزوماب (فیسجو) [14117]

التعليق على التشخيص : -----

قسم العلاج : الاورام

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشئون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: نجلاء رمضان عبدالهادي سليمان National ID: 27109220100581	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

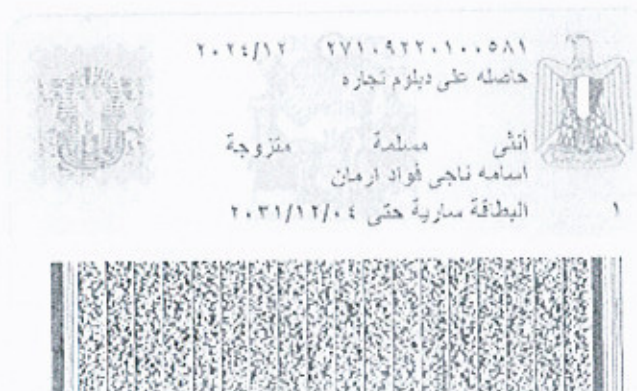
نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي (Phesgo (Subcutaneous trastuzumab and pertuzumab

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٢٥٥
٢٠٥٥

ملف رقمي

٢٢٨١

١ رقم الضبط

MDT FORM

27/12/2025 G

Name	خلال محمد عبدالقادر	MRN	2255/2025
Age	64	ID	2710922010058
Menopausal status	post -	Phone	01113537878
NRS SCORE		Marital status	m
BMI	average.	Family history	-ve
CO morbidity	HA TAHBSO.	REASON OF MDT	Surgeon

Clinical Data

BX:

R

L

ER:

318

PR:

318

HER 2:

43

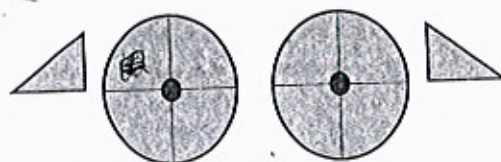
Ki67:

3090

Sonammammography:

2-t breast mass 1.7x1.8 cm.

another 1.1x1.4 cm 80-7x0.9



Mets Workup:

PST/CT → free

CTNM
10

received NACTH 6 x TCHP last 31/12/2025

Recommendations:

for Surgeon BCS + SLNB alba
we're laboratory
good response

MDT OUTCOME & PLAN

for wine localizer
at BCS SLNB

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

[Signature]

[Signature]

Name:	نجلاء رمضان عبدالهادي سليمان	
MR. :	3381	
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	
	رقم طبي:	
بقسم:		
إيماء للطلب المقدم من		
ننشر بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	
تاريخ وساعة الدخول :		
وبعد الكشف والفحوصات والمتابعة وجد انها		
PREMENOP RT BREAST CANCER CT1 NO MO CLIPS INSERTED ER3/8 PR3/8 HER2 3+REC 6 TCHP FOR C7 PHESGO SURGERY PENDING new mdt 28/2 = pt underwent rt BCS + SLNB post NACT YPT0 YPN0 FOR ADJ PHESGO + PORT + FEMARA X 5 YRS PENDING PHESGO, ECHO 10/2 67% FOR FEMARA + SUPPORTIVE PT0 NO M0 ADJUVANT PHESGO +FEMARA 1/C9 PHESGO (C10 +C11 IN PH) 2/EF 2/2026:67% Pharmacy Visit For C10 Phesgo + Femara+ Supp ECHO 2/2026 ...67% Pharmacy Visit For C11 Phesgo + Femara+ Supp ECHO 2/2026 ...67% clinical pharmacy note : phesgo wasn't available so for c 11 phesgo phesgo wasn't available so for c 11 phesgo TODAY + FEMARA + SUPPORTIVE TTT ECHO 17/5/2026 >> EF 64 % today for c12 phesgo, femara + supp echo 64% ef 17/5 for c15 phesgo and femara mri multiple disc lesions d12 weding and marrow signal for radiological correlation dental ex free for echo next time ef 57%1/9 cc dyspnea fatigue ,new labs pebding radiological revision of mri dl pending for new cts bs for c16 phesgo The patient has completed 16 cycles of Phesgo to date.		
تحريرا في ,		
	الوقت :	
20-09-2026		التاريخ :
dina makram		الطبيب المعالج :
يعتمد ,		

Patient Name :	الأستاذة / نجلاء رمضان عبد الهادي سليمان	Request No. :	13001827
Patient No. :	27109220100581	Request Date :	12/08/2025
Ref No. :	SR-1104/25	Verification Date :	14/08/2025
Sex :	Female	Financial Category :	جوستاف رومي - نفقة الدولة
Age :	53 Y	Department :	Out Patient
Referring Doc :		Registered by :	Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Right breast mass.

Preparation of specimen
Block & slide revision.

Gross examination
One block & 5 slides labelled by patient's name in arabic were received.

Microscopic examination

Histological type: Invasive duct carcinoma.

Grade (differentiation + nuclear pleomorphism + mitosis): (3+3+2) Grade 3.

DCIS: Absent

Angioinvasion: Absent.

Perineural invasion: Absent

Microcalcification : Absent .

PT (Positive control showed appropriate reactivity) :

ER: Weak nuclear reaction in 2% tumor cells (Allred score 3/8)

PR: Weak nuclear reaction in 10 % tumor cells (Allred score 3/8)

HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)

Ki 67: 30%

Diagnosis
Right breast mass, block & slide revision :
Invasive duct carcinoma, Grade 3

ER:

ER: Positive(score 3/8).

PR: Positive (score 3/8).

Verified by :

Dr.Heba Hasan



ent Name : الأستاذة / نجلاء رمضان عبد الهادي سليمان	Request No. : 13001827
ent No. : 27109220100581	Request Date : 12/08/2025
No. : SR-1104/25	Verification Date : 14/08/2025
: Female	Financial Category : نفقة الدولة - جومات روسي
: 53 Y	Department : Out Patient
arreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

ER2: Positive (score 3+).
i67: 30 %

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey .

Verified by :
Dr.Heba Hasan



Referred by: Prof. Amaal Lotefy Mohamed Al Gazzar

Date: 02/07/2025

Elastography revealed :

- Scattered mixed densities of glandular condensation are seen involving both breasts.
- Right breast UOQ lobulated outline dense lesion with pleomorphic microcalcifications involving a sector around lesion, the lesion is seen in ultrasound as solid hypoechoic mass 13x11 mm in diameters.
- No left speculated masses or pathological microcalcification.
- Normal skin thickness & contour of both breasts.
- No left breast solid or cystic masses.
- Bilateral small sized likely non-specific axillary lymph nodes.

OPINION :
Bilateral mild mammary fibroadenosis. left breast ,BIRADS 2.
Right breast UOQ suspicious mass and pleomorphic microcalcifications ,
BIRADS 4C , biopsy is recommended.

Kindly accept my best regards,
Dr. Naglaa Abdel Razek, M.D.

BIRADS 0 : Non-informative study.	0 %
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	2 %
BIRADS 3 : Probably benign.	50 %
BIRADS 4 : Probably malignant	
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	



S C A H

Name: Naglaa Ramadan Abd Al Hady

ID: 2131127

Referred by: Prof. Wael Samir Makkar

Date: 03/08/2025

High Field Open (1.5T) Dynamic MR-Mammography of Both Breasts

Technique :

- Axial T2 (plate 1). Axial STIR (plate 2).
- Sagittal postcontrast 3D TFE of the right breast (plate 3).
- Sagittal postcontrast 3D TFE of the left breast (plate 4).
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction (plate 5). Time intensity curves (plate 6).

Findings:

- Bilateral glandular condensation and evidences of bilateral homogenous glandular enhancement.
- Right breast UOQ 10:00 location zone B lobulated outline dense lesion with pleomorphic microcalcifications involving a sector around lesion , the mass measures 17x18mm in diameters , Associated two similar smaller lesions measuring 11x14 mm and 7x9 mm in diameters in the 10:00 location distal to the index mass , the area between the index mass to the posterior third of the breast, Zone C is associated with clumped nodular and ductal enhancement , the lesions are low signal in T2 and STAIR with intense mass like enhancement. The kinetic data obtained indicates a malignant pattern , rapid rise followed by wash out and plateau , type 2& 3 curves .
- No left breast masses and no architecture distortion.
- Normal skin thickness and contour of both breasts .
- There is multiple likely non-specific bilateral axillary lymphadenopathy.

OPINION :

Right breast UOQ multifocal carcinoma with segment of clumped enhancement corresponding to sector of DCIS ,categorized as BIRADS 5 based on the morphological and the kientic data.

Normal study of the left breast , BIRADS 1.

Prof. Naglaa Abdel Razek.MD



S C A N

Name: Naglaa Ramadan Abd Al Hady
Referred by: Prof. Wael Samir Makkar

ID: 2131127
Date: 03/08/2025

High Field Open (1.5T) Dynamic MR-Mammography of Both Breasts

Technique :

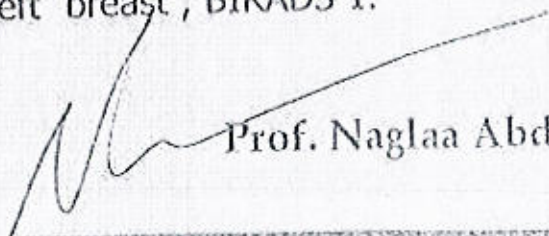
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- No left breast masses and no architecture distortion.
- Normal skin thickness and contour of both breasts.
- There is multiple likely non-specific bilateral axillary lymphadenopathy.

OPINION :

Right breast UOQ multifocal carcinoma with segment of clumped enhancement corresponding to sector of DCIS, categorized as BIRADS 5 based on the morphological and the kinetic data.
Normal study of the left breast, BIRADS 1.


Prof. Naglaa Abdel Razek.MD



S C A N

Name: *Naglaa Ramadan Abd Al Hady*
Referred by: *Prof. Wael Samir Mokker*

ID: *2131127*
Date: *05/08/2025*

¹⁸F-FDG PET/CT examination

Clinical Indication:

- Patient with history of pathologically proven right breast cancer.

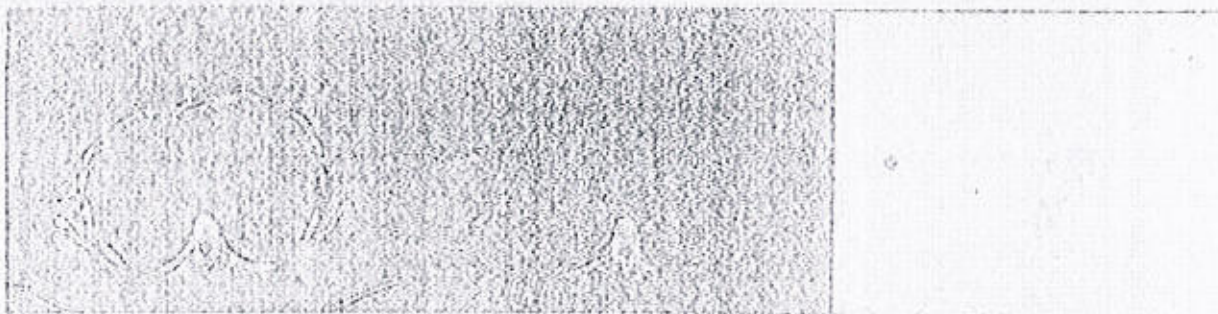
Administered activity: 259 MBq, ¹⁸F-FDG B.C.L.: 103 mg/dL

Examination protocol:

- A Whole body PET/CT study was obtained from skull vertex to the mid thigh using a low dose non contrast CT for attenuation correction
- Non-contrast CT examination of the same region.

I. PET/CT Findings:

- Right breast:



- Increased FDG uptake by right upper outer quadrant nodule with spiculated outline, SUVmax 6.2 and measures 2.1 x 1.5 cm. The lesion is surrounded by tiny satellite nodules and nodular densities demonstrating low grade FDG activity, SUVmax 2.1.
- No chest wall infiltration. No skin thickening.
- Left breast: no FDG avid lesions.